SACTCHECK · NCCP 00372

My complete treatment handbook

Nintedanib + docetaxel

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

Contents

1. Your three-page treatment companion — page 2
2. Full treatment reference: medicines, schedule, tests, symptoms and follow-up — page 5
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SACTCheck v0.84.31 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

Nintedanib + docetaxel

To help control adenocarcinoma lung cancer after first-line chemotherapy; treatment benefit and side effects are reviewed together.

My overall treatment

Day 1: docetaxel drip, with no nintedanib. Days 2–21: nintedanib capsules twice daily. Repeat every 21 days for up to 6 cycles (about 18 weeks without delays); treatment may stop earlier. A later tablet-only phase needs a new team-confirmed plan.

Combination phase
Day 1 docetaxel; Days 2–21 nintedanib twice daily. Up to 6 cycles.

Possible tablet-only phase
Only after at least 4 completed combination cycles and team approval: nintedanib every day in 28-day cycles. This change is not automatic.

A typical 21-day cycle

Day 1	Day 2–21
Docetaxel day	Nintedanib at home

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Blood count, kidney/liver tests and BP** Required by regimen	Before treatment; confirm allergy, medicines and pregnancy-prevention advice
During	**Blood count, kidney/liver tests** Required by regimen	Before each 21-day cycle; blood count also around the expected low-count period
During	**Symptoms and blood pressure** Required by regimen	BP periodically/as needed; extra blood or liver tests for symptoms or risk
After / review	**Phase, blood counts and scans** Source blood-count check; other plan team-agreed	After the last combination cycle: blood count as indicated; continuation requires review; scans/ markers individually planned

Between treatments

- Diarrhoea, nausea and fatigue may occur at home. Call early if drinking becomes difficult or symptoms persist.
- Low blood counts can occur between visits. The team will set an extra blood-test date; there is no guaranteed safe day.

Bring your medicine list and report new symptoms before dosing. Confirm test appointments, the current phase and daytime / out-of-hours contact plan.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Nerves / neurological symptoms

Both medicines

Report tingling; new severe headache, confusion, seizure or visual change is urgent.

3 Blood vessels / blood pressure

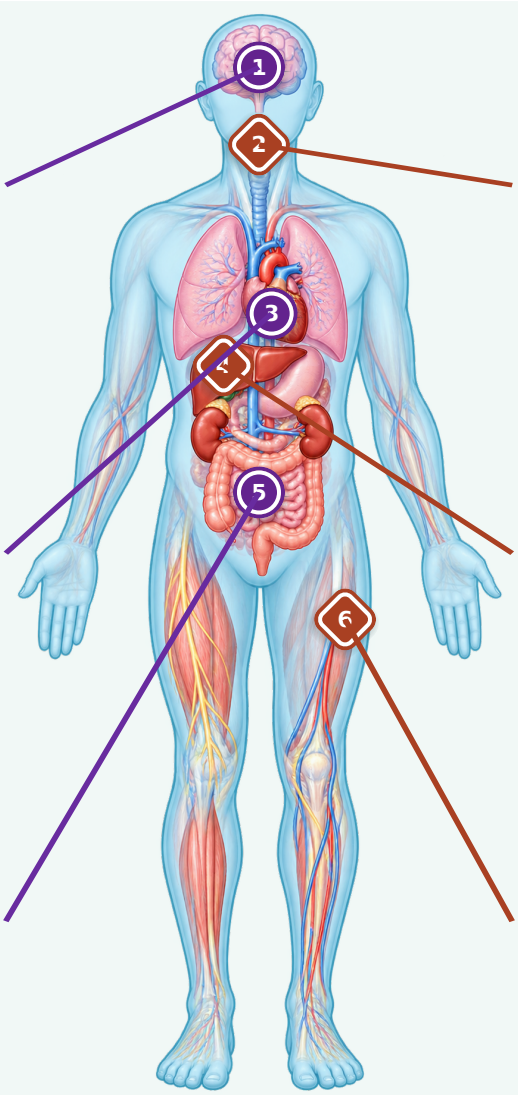
Nintedanib

Chest pain, sudden breathlessness, bleeding or a painful swollen leg needs urgent help.

5 Bowels / sickness

Both medicines

Call early for diarrhoea or vomiting; severe pain needs urgent assessment.



2 Mouth

Docetaxel

Report ulcers or pain that limits eating or drinking.

4 Liver

Nintedanib and docetaxel

Attend liver tests; report yellow skin/eyes or dark urine.

6 Blood counts / infection

Docetaxel with nintedanib

Fever, shaking chills or sudden illness needs immediate oncology advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

Keep your treatment details to hand

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Nintedanib + docetaxel

21-day cycle: docetaxel Day 1; nintedanib twice daily Days 2–21. Up to 6 cycles.

Phase: _____ Planned cycles: 6

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials
C4	**C5**	**C6**
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

Hospital / team: _____ Backup contact route: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

NCCP 00372 · VERSION 6

Your treatment, explained

Nintedanib + docetaxel

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Treatment stages
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

Three medicines. Different jobs.

To help control adenocarcinoma lung cancer after first-line chemotherapy; treatment benefit and side effects are reviewed together.

TAXANE CHEMOTHERAPY

Docetaxel

Docetaxel



Stabilises microtubules

Docetaxel prevents normal rearrangement of the cell structures needed for division. It affects cancer cells and can also suppress blood cells, irritate nerves and cause fluid retention.

Stabilises microtubules → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Fever or feeling suddenly unwell needs urgent advice. Tell the nurse immediately about infusion symptoms or pain.

Sources: nccp

ORAL ANGIOKINASE INHIBITOR

Nintedanib

Nintedanib



Blocks VEGF, FGF and PDGF receptor signalling

Nintedanib blocks several signals involved in the growth of blood vessels that support a tumour. Its effects can also cause diarrhoea, liver changes, bleeding and wound-healing problems.

Blocks VEGF, FGF and PDGF receptor signalling → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Call early for diarrhoea or poor fluid intake; report bleeding, yellow eyes, dark urine, severe tummy pain or new neurological symptoms urgently.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Treatment → review → next steps

COMBINATION PHASE

Combination phase

1–6

Day 1 docetaxel; Days 2–21 nintedanib twice daily. Up to 6 cycles.

POSSIBLE TABLET-ONLY PHASE

Possible tablet-only phase

Review

Only after at least 4 completed combination cycles and team approval: nintedanib every day in 28-day cycles. This change is not automatic.

Your team confirms each stage and discusses any changes with you.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My treatment and safety checks

Routine safety tests are separate from scans and tumour markers, which follow your individual plan. No universal imaging or tumour-marker interval is specified.

Check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment	Confirm combination plan	Day 1 docetaxel; Days 2–21 nintedanib	Capsules twice daily	After≥4 cycles, review possible continuation	No automatic phase switch
Blood count	Baseline	Before each cycle	Around nadir / if unwell	After last combination as indicated	Follow abnormalities
Kidneys / liver / BP	Baseline bloods and BP	Bloods each cycle	BP as indicated; extra tests if needed	Check tolerance before change	Liver monthly if oral continuation
Scans / other care	Individual disease plan	No fixed scan interval	Report symptoms early	Assess benefit	Agreed follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00372 v6 · official regimen · Vargatef current EMA product information

THE WHOLE TREATMENT JOURNEY

Nintedanib + docetaxel: treatment and checks

Adenocarcinoma NSCLC · combination phase

Day 1: docetaxel drip, with no nintedanib. Days 2–21: nintedanib capsules twice daily. Repeat every 21 days for up to 6 cycles (about 18 weeks without delays); treatment may stop earlier. A later tablet-only phase needs a new team-confirmed plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment NCCP 00372 · SMPC	Confirm combination plan	Day 1 docetaxel; Days 2–21 nintedanib	Capsules twice daily	After≥4 cycles, review possible continuation	No automatic phase switch
Blood count NCCP 00372 · SMPC	Baseline	Before each cycle	Around nadir / if unwell	After last combination as indicated	Follow abnormalities
Kidneys / liver / BP NCCP 00372 · SMPC	Baseline bloods and BP	Bloods each cycle	BP as indicated; extra tests if needed	Check tolerance before change	Liver monthly if oral continuation
Scans / other care NCCP 00372 · SMPC	Individual disease plan	No fixed scan interval	Report symptoms early	Assess benefit	Agreed follow-up

What each check is for

1

Prepare

Before treatment

To help control adenocarcinoma lung cancer after first-line chemotherapy; treatment benefit and side effects are reviewed together.

Ask / act: Confirm the aim, phase, prescription, tests and contacts.

Sources: NCCP 00372

2

Review

Before each cycle / at agreed reviews

Check benefit and tolerability

Ask / act: Report symptoms and confirm the next plan.

Sources: NCCP 00372

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

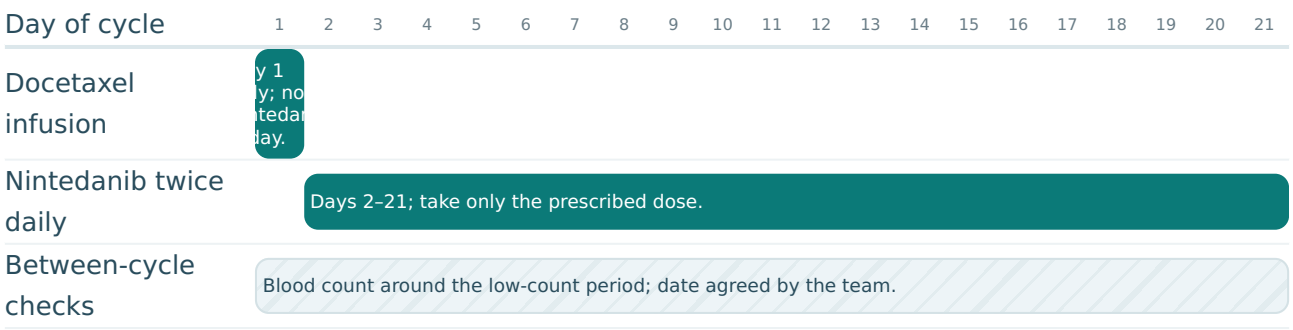
This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.30

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Day 1: docetaxel drip, with no nintedanib. Days 2–21: nintedanib capsules twice daily. Repeat every 21 days for up to 6 cycles (about 18 weeks without delays); treatment may stop earlier. A later tablet-only phase needs a new team-confirmed plan.



Symptoms can occur at any time: Side effects and infection do not follow a guaranteed day-by-day risk curve. Contact the team when symptoms occur.

Changes for safety: Treatment may be delayed, reduced or stopped. Do not make up missed doses or change your tablets yourself.

Why these bands stay broad: Relative days without personal dates. The team confirms actual treatment and test dates. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

There are no guaranteed “safe days”. Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

Read the whole cycle as text

Day 1: Docetaxel day

Docetaxel infusion after checks. Do not take nintedanib today. Follow the dexamethasone prescription. Use the current prescription and contact plan.

Day 2–21: Nintedanib at home

Take the prescribed capsule dose twice daily about 12 hours apart. Continue the agreed monitoring and support plan. Use the current prescription and contact plan.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Dexamethasone before docetaxel

Take the prescribed steroid course from the day before docetaxel. If missed, contact the unit before attending; do not invent a catch-up dose.



STEP 2

Infection prevention

The team should consider preventive G-CSF injections and arrange blood counts around the low-count period. Fever needs immediate advice.



STEP 3

Capsules

Take nintedanib about 12 hours apart with food; swallow whole. Do not open, chew or crush. If missed, take the next scheduled dose only; never double up.



STEP 4

Diarrhoea plan

Have the prescribed anti-diarrhoeal and fluid plan ready. Contact oncology early for diarrhoea, poor intake or vomiting; do not wait 7 days to seek help.



STEP 5

Medicine review

Check all medicines and supplements with oncology/pharmacy. Nintedanib has P-gp interactions; docetaxel has CYP 3 A interactions, including grapefruit concerns.



STEP 6

Pregnancy prevention

Discuss fertility and effective contraception. Nintedanib requires contraception during treatment and for at least 3 months after; follow the longest advice for your complete regimen. Vomiting/diarrhoea can reduce pill absorption.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00372 v6 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Dexamethasone before docetaxel

Take the prescribed steroid course from the day before docetaxel. If missed, contact the unit before attending; do not invent a catch-up dose.

2 Infection prevention

The team should consider preventive G-CSF injections and arrange blood counts around the low-count period. Fever needs immediate advice.

3 Capsules

Take nintedanib about 12 hours apart with food; swallow whole. Do not open, chew or crush. If missed, take the next scheduled dose only; never double up.

4 Diarrhoea plan

Have the prescribed anti-diarrhoeal and fluid plan ready. Contact oncology early for diarrhoea, poor intake or vomiting; do not wait 7 days to seek help.

5 Medicine review

Check all medicines and supplements with oncology/pharmacy. Nintedanib has P-gp interactions; docetaxel has CYP 3 A interactions, including grapefruit concerns.

6 Pregnancy prevention

Discuss fertility and effective contraception. Nintedanib requires contraception during treatment and for at least 3 months after; follow the longest advice for your complete regimen. Vomiting/diarrhoea can reduce pill absorption.

Sources: nccp · ncis · antiemetic

Understanding the evidence

The trial found a progression-free benefit overall and an overall-survival benefit in the prespecified adenocarcinoma subgroup.

The licensed/NCCP population is adenocarcinoma after first-line chemotherapy. Overall-population PFS and adenocarcinoma-subgroup OS are labelled separately; this does not establish an independent benefit from maintenance-only initiation.

Sources: trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Who confirms my next dose?

Your treating team checks the prescription, tests and symptoms. A completed-cycle stamp records past treatment; it does not authorise the next dose.

What if the plan changes?

Ask the team to update your phase, medicines, test appointments and cycle record. Use the updated prescription.

When are my scans?

Your team sets a response and follow-up plan for your cancer and symptoms. This document does not invent a fixed scan or tumour-marker timetable.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00372 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00372 v6 · official regimen
- Vargatef current EMA product information
- NCCP antiemetic classification v 6 and oral pathway
- NCIS medical oncology antiemetic medicines
- LUME-Lung 1 primary publication

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6913/372_Nintedanib_therapy.pdf

https://www.ema.europa.eu/en/documents/product-information/vargatef-epar-product-information_en.pdf

https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/24411639/

BASELINE → TREATMENT → FOLLOW-UP

My treatment and safety checks

Routine safety tests are separate from scans and tumour markers, which follow your individual plan. No universal imaging or tumour-marker interval is specified.

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THE WHOLE TREATMENT JOURNEY

Nintedanib + docetaxel: treatment and checks

Adenocarcinoma NSCLC · combination phase

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Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment NCCP 00372 · SMPC	Confirm combination plan	Day 1 docetaxel; Days 2–21 nintedanib	Capsules twice daily	After≥4 cycles, review possible continuation	No automatic phase switch
Blood count NCCP 00372 · SMPC	Baseline	Before each cycle	Around nadir / if unwell	After last combination as indicated	Follow abnormalities
Kidneys / liver / BP NCCP 00372 · SMPC	Baseline bloods and BP	Bloods each cycle	BP as indicated; extra tests if needed	Check tolerance before change	Liver monthly if oral continuation
Scans / other care NCCP 00372 · SMPC	Individual disease plan	No fixed scan interval	Report symptoms early	Assess benefit	Agreed follow-up

What each check is for

1 Prepare

Before treatment

To help control adenocarcinoma lung cancer after first-line chemotherapy; treatment benefit and side effects are reviewed together.

Ask / act: Confirm the aim, phase, prescription, tests and contacts.

Sources: NCCP 00372

2 Review

Before each cycle / at agreed reviews

Check benefit and tolerability

Ask / act: Report symptoms and confirm the next plan.

Sources: NCCP 00372

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.30

NCCP 00372 · clinician draft

Nintedanib + docetaxel

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Combination · docetaxel Day 1 and steroid premedication** — Low parenteral
  Docetaxel
- **Combination · nintedanib Days 2–21** — Minimal-to-low oral
  Nintedanib

https://sactcheck.com/patient/00372/combination/supportive-care.html

Nintedanib + docetaxel

NCCP 00372 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Dexamethasone before docetaxel

Take the prescribed steroid course from the day before docetaxel. If missed, contact the unit before attending; do not invent a catch-up dose.

STEP 2

Infection prevention

The team should consider preventive G-CSF injections and arrange blood counts around the low-count period. Fever needs immediate advice.

STEP 3

Capsules

Take nintedanib about 12 hours apart with food; swallow whole. Do not open, chew or crush. If missed, take the next scheduled dose only; never double up.

STEP 4

Diarrhoea plan

Have the prescribed anti-diarrhoeal and fluid plan ready. Contact oncology early for diarrhoea, poor intake or vomiting; do not wait 7 days to seek help.

STEP 5

Medicine review

Check all medicines and supplements with oncology/ pharmacy. Nintedanib has P-gp interactions; docetaxel has CYP 3 A interactions, including grapefruit concerns.

STEP 6

Pregnancy prevention

Discuss fertility and effective contraception. Nintedanib requires contraception during treatment and for at least 3 months after; follow the longest advice for your complete regimen. Vomiting/diarrhoea can reduce pill absorption.

Sources: NCCP 00372 v6 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

Nintedanib + docetaxel

A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Day 1: docetaxel drip, with no nintedanib. Days 2–21: nintedanib capsules twice daily. Repeat every 21 days for up to 6 cycles (about 18 weeks without delays); treatment may stop earlier. A later tablet-only phase needs a new team-confirmed plan.

These pages are reusable recording sheets; the prescribed course length is stated above. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital’s instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00372/combination/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

Nintedanib + docetaxel

Cycle number: _____ Start date: _____ Next review: _____

Record the actual phase, dose dates, symptoms and interruptions. A completed-cycle stamp is a record of treatment, not clearance for another dose.

Cycle 1 · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999

Nintedanib · Oral angiokinase inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Nintedanib + docetaxel

Cycle number: _____ Start date: _____ Next review: _____

Record the actual phase, dose dates, symptoms and interruptions. A completed-cycle stamp is a record of treatment, not clearance for another dose.

Cycle 2 · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999

Nintedanib · Oral angiokinase inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Nintedanib + docetaxel

Cycle number: _____ Start date: _____ Next review: _____

Record the actual phase, dose dates, symptoms and interruptions. A completed-cycle stamp is a record of treatment, not clearance for another dose.

Cycle 3 · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999

Nintedanib · Oral angiokinase inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Nintedanib + docetaxel

Cycle number: _____ Start date: _____ Next review: _____

Record the actual phase, dose dates, symptoms and interruptions. A completed-cycle stamp is a record of treatment, not clearance for another dose.

Cycle 4 · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999

Nintedanib · Oral angiokinase inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Nintedanib + docetaxel

Cycle number: _____ Start date: _____ Next review: _____

Record the actual phase, dose dates, symptoms and interruptions. A completed-cycle stamp is a record of treatment, not clearance for another dose.

Cycle 5 · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999

Nintedanib · Oral angiokinase inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Nintedanib + docetaxel

Cycle number: _____ Start date: _____ Next review: _____

Record the actual phase, dose dates, symptoms and interruptions. A completed-cycle stamp is a record of treatment, not clearance for another dose.

Cycle 6 · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999

Nintedanib · Oral angiokinase inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Nintedanib + docetaxel

21-day cycle: docetaxel Day 1; nintedanib twice daily Days 2–21. Up to 6 cycles.

Phase: _____ Planned cycles: 6

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials
C4	**C5**	**C6**
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

Hospital / team: _____ Backup contact route: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.31 · NCCP 00372 v6 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.